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Dermatite atópica

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O que é?

Dermatite atópica (ou eczema atópico) é um processo inflamatório crônico da pele caracterizado por lesões avermelhadas, que coçam muito e, às vezes, descamam. Geralmente, elas se localizam na face das crianças pequenas e nas dobras do joelho e cotovelo das crianças maiores e dos adultos. A dermatite atópica pode estar associada a outras atopias, como bronquite, asma e rinite, por exemplo.

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O que é?

Sabe-se, também, que alguns fatores de risco funcionam como gatilho das crises. Entre eles destacam-se: substâncias irritantes (poeira domiciliar, conservantes, produtos de limpeza e usados na lavagem das roupas), tecidos de lã e sintéticos, frio intenso, ambientes secos, calor e transpiração, estresse emocional.

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Maior orgão do corpo humano

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O que causa?

- Dieta
- Intestino - fígado
- Hidratação
- Estresse
- Alteração tireoide
- Alergias alimentares - leakgut
- Lácteos
- Glúten
- Ovo
- Soja
- Nozes, amêndoas
- Peixes
- Venenos na cozinha
- Vit D
- Aditivos,e industrializados

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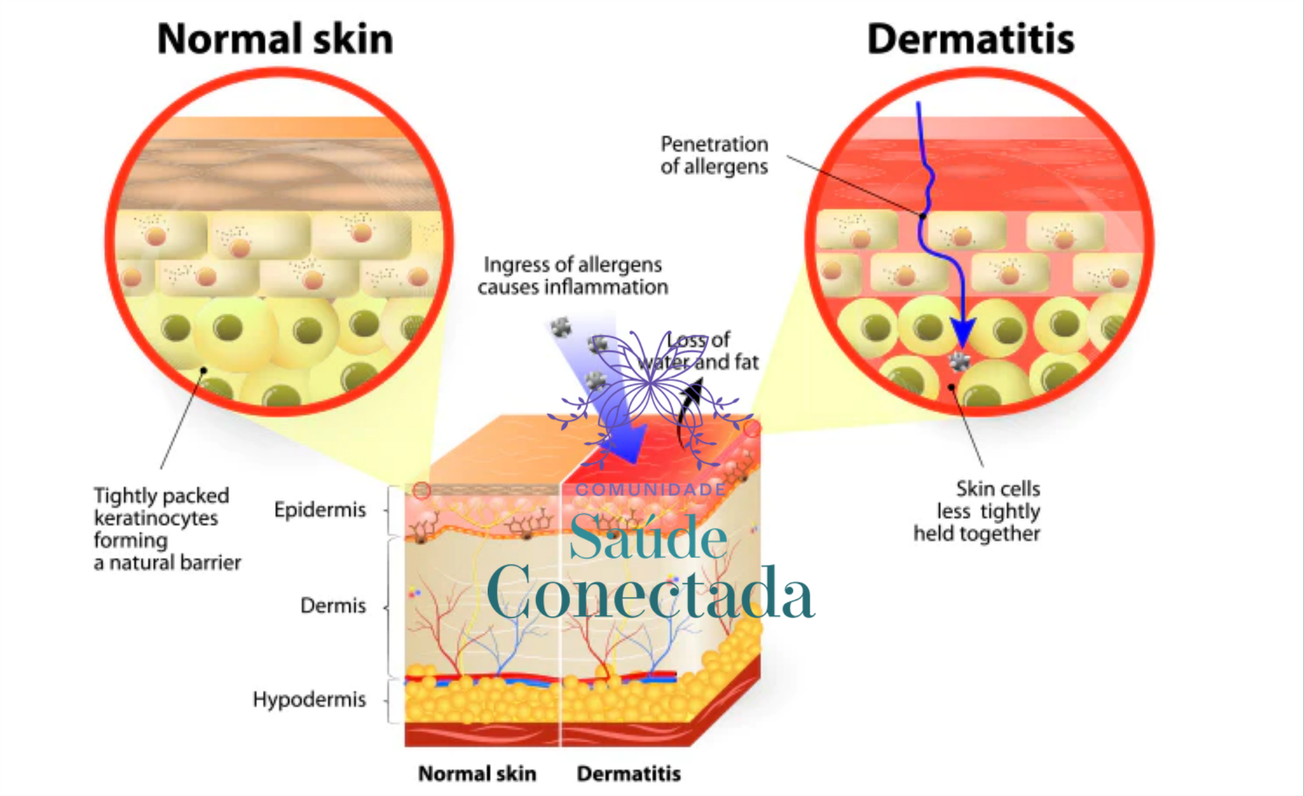

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Review > Curr Opin Allergy Clin Immunol. 2016 Oct;16(5):458-64.
doi: 10.1097/ACI.0000000000000308.

Atopic dermatitis: new evidence on the role of allergic inflammation

Annice Heratizadeh [1]

Affiliations + expand

PMID: 27490127 DOI: 10.1097/ACI.0000000000000308

Abstract

Purpose of review: Atopic dermatitis is a chronic relapsing inflammatory skin disease. In the presence of a complex genetic background, there is increasing evidence for the role of specific allergenic trigger factors in perpetuating skin inflammation in sensitized atopic dermatitis patients. In this review, clinical and in-vitro data so far published on allergen-induced adaptive immune responses in atopic dermatitis are summarized.

Recent findings: Emerging new data have been published particularly on adaptive immune responses to inhalant allergens in atopic dermatitis. In a randomized controlled study, the induction of a flare-up by grass pollen exposure in sensitized atopic dermatitis patients could be demonstrated for the first time. T cells directed to the two major allergens of house dust mite have been characterized to display a Th2, and moreover, a Th17 and Th2/Th17 phenotype in sensitized atopic dermatitis patients. With regard to microbial antigens, T cell-mediated immune responses directed to proteins of the species themselves can be observed - as has been published for Staphylococcus aureus and Malassezia spp. Beyond this, specific T-cell activation to cross-reacting human proteins might further trigger the disease in distinct patients. The role of 'autoallergic' phenomena in atopic dermatitis, because of human antigens without known cross-reactivity to environmental allergens, is currently under investigation as well.

Summary: Recent findings on immunological and clinical characteristics of adaptive immune responses to allergens in atopic dermatitis, but also on the identification of new, potentially relevant allergen sources might contribute to the development of effective treatment strategies 'customized' for allergic inflammation in atopic dermatitis in future.

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Microbiome in the Gut–Skin Axis in Atopic Dermatitis

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The microbiome is vital for immune system development and homeostasis. Changes in microbial composition and function, termed dysbiosis, in the skin and the gut have recently been linked to alterations in immune responses and to the development of skin diseases, such as atopic dermatitis (AD). In this review, we summarize the recent findings on the gut and skin microbiome, highlighting the roles of major commensals in modulating skin and systemic immunity in AD. Although our understanding of the gut-skin axis is only beginning, emerging evidence indicates that the gut and skin microbiome could be manipulated to treat AD.

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Intestino - alergia

Sistema imunológico

Inflamação

Doença de pele

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> FASEB J. 2022 Jun;36(6):e22334. doi: 10.1096/fj.202101800R.

IL-20 promotes cutaneous inflammation and peripheral itch sensation in atopic dermatitis

Zhiping Lu [1], Song Xiao [1], Weiwei Chen [1], Renkai Zhu [1], Hua Yang [1], Martin Steinhoff [2 3 4 5 6 7], Yanqing Li [1], Wenke Cheng [1], Xinrong Yan [1], Lianlian Li [1], Shanghai Xue [1], Ciara Larkin [8 9], Wenhao Zhang [1], Qianqian Fan [1], Ruizhen Wang [1], Jiafu Wang [8], Jianghui Meng [8 9]

Affiliations + expand

PMID: 35486004 PMCID: PMC9321592 DOI: 10.1096/fj.202101800R

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Abstract

Atopic dermatitis (AD) is a chronic skin disease, which is associated with intense itch, skin barrier dysfunction and eczematous lesions. Aberrant IL-20 expression has been implicated in numerous inflammatory diseases, including psoriasis. However, the role of IL-20 in AD remains unknown. Here, RNA-seq, Q-PCR, and immunocytochemistry were utilized to examine disease-driven changes of IL-20 and its cognate receptor subunits in skin from healthy human subjects, AD patients and murine AD-models. Calcium imaging, knockdown and cytokine array were used to investigate IL-20-evoked responses in keratinocytes and sensory neurons. The murine cheek model and behavioral scoring were employed to evaluate IL-20-elicited sensations in vivo. We found that transcripts and protein of IL-20 were upregulated in skin from human AD and murine AD-like models. Topical MC903 treatment in mice ear enhanced IL-20R1 expression in the trigeminal sensory ganglia, suggesting a lesion-associated and epidermal-driven mechanism for sensitization of sensory IL-20 signaling. IL-20 triggered calcium influx in both keratinocytes and sensory neurons, and promoted their AD-related molecule release and transcription of itch-related genes. In sensory neurons, IL-20 application increased TLR2 transcripts, implicating a link between innate immune response and IL-20. In a murine cheek model of acute itch, intradermal injection IL-20 and IL-13 elicited significant itch-like behavior, though only when co-injected. Our findings provide novel insights into IL-20 function in peripheral (skin-derived) itch and clinically relevant intercellular neuron-epidermal communication, highlighting a role of IL-20 signaling in the pathophysiology of AD, thus forming a new basis for the development of a novel antipruritic strategy via interrupting IL-20 epidermal pathways.

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> J Allergy Clin Immunol. 2022 Jun;149(6):1875-1898. doi: 10.1016/j.jaci.2022.03.010.
Epub 2022 Mar 23.

Neuroimmune communication regulating pruritus in atopic dermatitis

Martin Steinhoff [1], Fareed Ahmad [2], Atul Pandey [2], Angeliki Datsi [3], Ayda AlHammadi [4], Sara Al-Khawaga [4], Aysha Al-Malki [4], Jianghui Meng [5], Majid Alam [2], Joerg Buddenkotte [6]

Affiliations + expand

PMID: 35337846 DOI: 10.1016/j.jaci.2022.03.010

Free article

Abstract

Atopic dermatitis (AD) is a common, chronic-relapsing inflammatory skin disease with significant disease burden. Genetic and environmental trigger factors contribute to AD, activating 2 of our largest organs, the nervous system and the immune system. Dysregulation of neuroimmune circuits plays a key role in the pathophysiology of AD, causing inflammation, pruritus, pain, and barrier dysfunction. Sensory nerves can be activated by environmental or endogenous trigger factors, transmitting itch stimuli to the brain. On stimulation, sensory nerve endings also release neuromediators into the skin, contributing again to inflammation, barrier dysfunction, and itch. In addition, dysfunctional peripheral and central neuronal structures contribute to neuroinflammation, sensitization, nerve elongation, and neuropathic itch, thus chronification and therapy resistance. Consequently, neuroimmune circuits in skin and central nervous system may be targets to treat pruritus in AD. Cytokines, chemokines, proteases, lipids, opioids, and ions excite/sensitize sensory nerve endings, which not only induces itch but further aggravates/perpetuates inflammation, skin barrier disruption, and pruritus as well. Thus, targeted therapies for neuroimmune circuits as well as pathway inhibitors (eg, kinase inhibitors) may be beneficial to control pruritus in AD either in systemic and/or in topical form. Understanding neuroimmune circuits and neuronal signaling will optimize our approach to control all pathological mechanisms in AD, inflammation, barrier dysfunction, and pruritus.

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PRIMEIRO PASSO

Protocolo Desinflama Conecta

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J Nippon Med Sch. 2021 Jun 30;88(3):171-177. doi: 10.1272/jnms.JNMS.2021_88-317.
Epub 2021 Mar 9.

Nutrition and Atopic Dermatitis

Naoko Kanda [1], Toshihiko Hoashi [2], Hidehisa Saeki [2]

Affiliations + expand

PMID: 33692290 DOI: 10.1272/jnms.JNMS.2021_88-317

Free article

Abstract

Atopic dermatitis (AD) is a chronic eczematous disease characterized by T helper 2 (Th2) -shifted allergic immunity, skin barrier impairment, and pruritus. Oral intake of certain nutrients might help regulate AD. Serum 25-hydroxyvitamin D levels are often low in patients with AD, and oral vitamin D supplementation improves AD. Vitamin D increases regulatory T (Treg) cells, which promote tolerance to allergens and prevent allergic inflammation by inducing expression of filaggrin and cathelicidin in keratinocytes. Vitamin A strengthens Treg cells by inducing expression of forkhead box P3 and inhibits mediator release from mast cells and eosinophils. Serum levels of γ-linolenic acid and its metabolite, dihomo-γ-linolenic acid, are low in patients with AD, and oral γ-linolenic acid improves AD through anti-inflammatory prostaglandin D_1 and E_1 derived from dihomo-γ-linolenic acid. Eicosapentaenoic acid and docosahexaenoic acid ameliorate AD by suppressing production of leukotriene B_4, increasing ceramides in the stratum corneum, and through their metabolites, resolvin E1 and D1, which resolve inflammation. The probiotics Lactobacillus and Bifidobacteria improve the intestinal permeability barrier and induce Treg cells. Zinc levels in serum, hair, and erythrocytes are diminished in patients with AD. Zinc induces forkhead box P3 expression and increases Treg cells, and zinc-finger protein A20 suppresses nuclear factor-κB-dependent expression of inflammatory cytokines and cell-adhesion molecules. Oral supplementation of the above nutrients might have therapeutic or preventive roles in AD.

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Dieta de eliminação

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Você irá fazer a exclusão dos seguintes alimentos por 45 dias :

- Glúten
- Lácteos
- Milho
- Soja
- Oleaginosas
- Leguminosas
- Cafeína
- Solanáceas (batata, tomate, berinjela, pimenta e o pepino)
- Cítricos
- Ovos
- Fermentados

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Após, você fará a introducão de 1 grupo por vez, 3 x ao dia

Ex:
Dia 1 - manteiga no café da manhã, queijo parmesão no almoço e 1 iogurte no lanche da tarde.
Dia 2 - voltar para o cardápio sem lácteos
Dia 3 - Manter o cardápio sem lácteos
Dia 4 - Observar a reação e anotar. Manter o dia 4 sem lácteos
Dia 5 - Se ficou tudo bem, manter os lácteos com moderação e inserir ou grupo de alimentos. Por ex: frutas cítricas.

Caso tenha reação, excluir do seu cardápio o alimento por 3 meses e testar novamente após esse tempo.

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Suplementos

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CSC

- Probióticos (6 meses)
- Óleo de borrage / prímula
- Chá oolong, tulsi, cúrcuma, ginseng, centella asiática, erva de Sao Cristovao, licorice, folha de amora
- Vit. A, E, Zn
- Vit. D

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CSC

- Polifenóis
- Silimarina
- Quassia
- Aveia
- Óleo de cânfora

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J Dermatolog Treat. 2019 Dec;30(8):785-789. doi: 10.1080/09546634.2019.1573306.
Epub 2019 Feb 3.

Black tea dressings – a rapidly effective treatment for facial dermatitis

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Affiliations + expand

PMID: 30668184 DOI: 10.1080/09546634.2019.1573306

Abstract

Background: Facial dermatitis is a common dermatologic condition. It is usually treated with either topical corticosteroids or calcineurin inhibitors. However, the use of these drugs is limited by side effects, including skin atrophy, local immunosuppression and possible oncogenicity. **Objective:** The aim of the study was to evaluate the use of wet dressings with black tea for treating facial dermatitis. **Methods:** We performed a prospective, open, uncontrolled before-after study enrolling 22 patients with atopic or contact facial dermatitis who were treated with black tea dressings and an emollient cream over 6 days. Disease severity was assessed using the (1) Facial Eczema Area and Severity Index, (2) Visual analog scale for pruritus, (3) Investigator`s Global Assessment score, and (4) Patient's Self-Assessment score. The study was registered at www.ClinicalTrials.gov as NCT02941432. **Results:** A dramatic and highly significant reduction of all four disease activity scores occurred within the first 3 days of treatment and the patients continued to improve between days 3 and 6. No side effects were observed. **Conclusion:** Black tea dressings represent an effective treatment option for facial dermatitis. Its advantages include lack of side effects, low cost, and easy availability.

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Front Nutr. 2022; 9: 953646.
Published online 2022 Aug 9. doi: 10.3389/fnut.2022.953646
PMCID: PMC9395728
PMID: 36017227

Ameliorative effects of epigallocatechin-3-gallate nanoparticles on 2,4-dinitrochlorobenzene induced atopic dermatitis: A potential mechanism of inflammation-related necroptosis

Mengguo Han, Xue Wang, Jian Wang, Dongcen Lang, Xiaohua Xia, Yongfang Jia, and Ying Chen*

Atopic dermatitis (AD) is a common autoimmune and chronic inflammatory cutaneous disease with a relapsing-remitting course. Necroptosis is a regulated necrotic cell death mediated by receptor-interacting protein 1 (RIP1), receptor-interacting protein 3 (RIP3), and mixed lineage kinase domain-like pseudokinase (MLKL), which is activated by tumor necrosis factor-α (TNF-α). However, the mechanism and the role of necroptosis have not been delineated in AD progression. (-)-Epigallocatechin-3-gallate (EGCG), the main biological activity of tea catechin, is well known for its beneficial effects in the treatment of skin diseases. Here, PEG-PLGA-EGCG nanoparticles (EGCG-NPs) were formulated to investigate the bioavailability of EGCG to rescue cellular injury following the inhibition of necroptosis after AD. 2,4-dinitrochlorobenzene (DNCB) was used to establish AD mouse models. As expected, topically applied EGCG-NPs elicited a significant amelioration of AD symptoms in skin lesions, including reductions in the ear and skin thickness, dermatitis score, and scratching behavior, which was accompanied by redox homeostasis restored early in the experiment. In addition, EGCG-NPs significantly decreased the expression of inflammatory cytokines like TNF-α, interferon-γ (IFN-γ), interleukin-4 (IL-4), interleukin-6 (IL-6), and interleukin-17A (IL-17A) in a time-dependent manner than those of in AD group. As a result, the overexpression of RIP1, RIP3, and MLKL in the entire epidermis layers was dramatically blocked by EGCG-NPs, as well as the expression ofphosphorylated p38 (p-p38), extracellular signal-regulated kinase 1 (ERK1), and extracellular signal-regulated kinase 2 (ERK2). These findings promote that EGCG-NPs formulation represents a promising drug-delivery strategy for the treatment of AD by maintaining the balance of Th1/Th2 inflammation response and targeting necroptosis.

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> Mycobiology. 2012 Jun;40(2):124-8. doi: 10.5941/MYCO.2012.40.2.124. Epub 2012 Jun 29.

Treatment of Atopic Dermatitis Associated with Malassezia sympodialis by Green Tea Extracts Bath Therapy: A Pilot Study

Hyun Kyu Kim [1], Hui Kyoung Chang, Seok Yun Baek, Jin Oh Chung, Chan Su Rha, So Young Kim, Beom Joon Kim, Myeung Nam Kim

Affiliations + expand

PMID: 22870055 PMCID: PMC3408302 DOI: 10.5941/MYCO.2012.40.2.124

Free PMC article

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Abstract

Multiple treatment modalities, including topical and systemic corticosteroid and phototherapy, have been used in treatment of patients with atopic dermatitis. However, long-term corticosteroid therapy may have various adverse effects. The purpose of this study was to investigate the therapeutic efficacy and safety of bath therapy using green tea extracts for treatment of patients with atopic dermatitis. A total of four patients with atopic dermatitis were enrolled in this study. A Malassezia multiplex detection kit was used in performance of multiplex PCR on clinical isolates, which confirmed Malassezia sympodialis. Subjects underwent treatment with bath therapy using green tea extracts three times per wk for a period of 4 wk. Assessment using the scoring atopic dermatitis (SCORAD) index, the visual analogue scale for pruritus, and transepidermal water loss was performed weekly. Laboratory tests were performed before and after treatment. All patients showed marked improvement on the mean SCORAD and visual analogue scale, and a significant decrease in the mean values of serum eosinophil counts was observed after treatment. Bath therapy with green tea extract is an effective, safe, and nonsteroidal therapy for treatment of patients with atopic dermatitis associated with Malassezia sympodialis.

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Review > Ann Intern Med. 1983 May;98(5 Pt 1):641-53. doi: 10.7326/0003-4819-98-5-641.

The health consequences of caffeine

P W Curatolo, D Robertson

PMID: 6342491 DOI: 10.7326/0003-4819-98-5-641

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Abstract

Acutely administered caffeine modestly increases blood pressure, plasma catecholamine levels, plasma renin activity, serum free fatty acid levels, urine production, and gastric acid secretion. It alters the electroencephalographic spectrum, mood, and sleep patterns of normal volunteers. Chronic caffeine consumption has no effect on blood pressure, plasma catecholamine levels, plasma renin activity, serum cholesterol concentration, blood glucose levels, or urine production. Caffeine does not appear to be useful for increasing the motility of hypomotile sperm in artificial insemination or in the therapy of minimal brain dysfunction, cancer, or Parkinson's syndrome, but it may be effective as a topical treatment of atopic dermatitis and as systemic therapy for neonatal apnea. Caffeine does not seem to be associated with myocardial infarction; lower urinary tract, renal, or pancreatic cancer; teratogenicity; or fibrocystic breast disease. The role of caffeine in the production of cardiac arrhythmias or gastric or duodenal ulcers remains uncertain.

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Biomedicines. 2020 Feb; 8(2): 27.
Published online 2020 Feb 8. doi: 10.3390/biomedicines8020027
PMCID: PMC7168306
PMID: 32046246

New Herbal Biomedicines for the Topical Treatment of Dermatological Disorders

Julia Hoffmann, Fabian Gendrisch, Christoph Mathis Schempp, and Ute Wölfle*

Author information · Article notes · Copyright and License information Disclaimer

Abstract

Go to:

Herbal extracts and isolated plant compounds play an increasing role in the treatment of skin disorders and wounds. Several new herbal drugs, medicinal products and cosmetic products for the treatment of various skin conditions have been developed in recent years. In this nonsystematic review, we focus on herbal drugs that were tested in controlled clinical studies or in scientifically sound preclinical studies. The herbal biomedicines are intended to treat atopic dermatitis (St. John's wort, licorice, tormentil, bitter substances, evening primrose), psoriasis (araroba tree, lace flower, barberry bark, indigo, turmeric, olibanum, St. John's wort), actinic keratosis (birch bark, petty spurge), herpes simplex (lemon balm, sage and rhubarb), rosacea (green tea, licorice, tormentil) and acne vulgaris (tea tree oil, green tea, hop), or to improve photo protection (green tea, Dyer's weed, cocoa tree, carotinoids, licorice), aesthetic dermatology (licorice, pine bark, gotu kola) and wound healing (birch bark, onion).

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Erva de São João
Tormentil
Licorice
Substâncias amargas
Prímula

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Randomized Controlled Trial > Nutrients. 2022 Oct 25;14(21):4470. doi: 10.3390/nu14214470.

Efficacy and Safety of Fig (*Ficus carica* L.) Leaf Tea in Adults with Mild Atopic Dermatitis: A Double-Blind, Randomized, Placebo-Controlled Preliminary Trial

Tatsuya Abe [1], Yukari Koyama [1], Kosaku Nishimura [1], Aya Okiura [1], Toru Takahashi [1]

Affiliations + expand

PMID: 36364740 PMCID: PMC9658579 DOI: 10.3390/nu14214470

Free PMC article

Abstract

Atopic dermatitis (AD) is a chronic, recurrent pruritic skin disease with repeated remissions and exacerbations. Various factors, such as allergies, skin conditions and lifestyle, combine to cause AD, making it difficult to cure completely. Although AD symptoms are suppressed with medications, this is a long-term effort and burden on patients. Thus, safer drugs and alternatives are needed. We previously found that consumption of tea prepared from fig (*Ficus carica* L.) leaves alleviated allergy and AD symptoms in cultured cells and animals. Therefore, here, we conducted a double-blind, randomized, controlled study in patients with mild AD to evaluate the safety and AD-relieving effects of prolonged consumption of fig leaf tea. Positive effects of fig leaf tea consumption were confirmed in 14 of 15 participants. Eczema Area and Severity Index values were significantly lowered in the fig leaf tea-treated group than in the placebo-treated group. The effect weakened 4 weeks after the end of the intervention, suggesting that continued intake of fig leaf tea was effective. Further assessments confirmed the safety of fig leaf tea consumption and revealed no variations that might pose a health hazard. Therefore, we postulate that fig leaf tea is a natural and safe therapeutic option for AD.

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Review > Int J Mol Sci. 2020 Jan 23;21(3):741. doi: 10.3390/ijms21030741.

Omega-3 Versus Omega-6 Polyunsaturated Fatty Acids in the Prevention and Treatment of Inflammatory Skin Diseases

Anamaria Balić [1], Domagoj Vlašić [2], Kristina Žužul [3], Branka Marinović [1], Zrinka Bukvić Mokos [1]

Affiliations + expand

PMID: 31979308 PMCID: PMC7037798 DOI: 10.3390/ijms21030741

Free PMC article

Abstract

Omega-3 (ω-3) and omega-6 (ω-6) polyunsaturated fatty acids (PUFAs) are nowadays desirable components of oils with special dietary and functional properties. Their therapeutic and health-promoting effects have already been established in various chronic inflammatory and autoimmune diseases through various mechanisms, including modifications in cell membrane lipid composition, gene expression, cellular metabolism, and signal transduction. The application of ω-3 and ω-6 PUFAs in most common skin diseases has been examined in numerous studies, but their results and conclusions were mostly opposing and inconclusive. It seems that combined ω-6, gamma-linolenic acid (GLA), and ω-3 long-chain PUFAs supplementation exhibits the highest potential in diminishing inflammatory processes, which could be beneficial for the management of inflammatory skin diseases, such as atopic dermatitis, psoriasis, and acne. Due to significant population and individually-based genetic variations that impact PUFAs metabolism and associated metabolites, gene expression, and subsequent inflammatory responses, at this point, we could not recommend strict dietary and supplementation strategies for disease prevention and treatment that will be appropriate for all. Well-balanced nutrition and additional anti-inflammatory PUFA-based supplementation should be encouraged in a targeted manner for individuals in need to provide better management of skin diseases but, most importantly, to maintain and improve overall skin health.

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Review > Nutrition. 2010 Jul-Aug;26(7-8):708-18. doi: 10.1016/j.nut.2009.10.014.

Borage oil in the treatment of atopic dermatitis

Rachel H Foster [1], Gil Hardy, Raid G Alany

Affiliations + expand

PMID: 20579590 DOI: 10.1016/j.nut.2009.10.014

Abstract

Nutritional supplementation with omega-6 essential fatty acids (omega-6 EFAs) is of potential interest in the treatment of atopic dermatitis. EFAs play a vital role in skin structure and physiology. EFA deficiency replicates the symptoms of atopic dermatitis, and patients with atopic dermatitis have been reported to have imbalances in EFA levels. Although direct proof is lacking, it has been hypothesized that patients with atopic dermatitis have impaired activity of the delta-6 desaturase enzyme, affecting metabolism of linoleic acid to gamma-linolenic acid (GLA). However, to date, studies of EFA supplementation in atopic dermatitis, most commonly using evening primrose oil, have produced conflicting results. Borage oil is of interest because it contains two to three times more GLA than evening primrose oil. This review identified 12 clinical trials of oral or topical borage oil for treatment of atopic dermatitis and one preventive trial. All studies were controlled and most were randomized and double-blind, but many were small and had other methodological limitations. The results of studies of borage oil for the treatment of atopic dermatitis were highly variable, with the effect reported to be significant in five studies, insignificant in five studies, and mixed in two studies. Borage oil given to at-risk neonates did not prevent development of atopic dermatitis. However, the majority of studies showed at least a small degree of efficacy or were not able to exclude the possibility that the oil produces a small benefit. Overall, the data suggest that nutritional supplementation with borage oil is unlikely to have a major clinical effect but may be useful in some individual patients with less severe atopic dermatitis who are seeking an alternative treatment. Which patients are likely to respond cannot yet be identified. Borage oil is well tolerated in the short term but no long-term tolerability data are available.

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Evening Primrose

No authors listed

In: Drugs and Lactation Database (LactMed®) [Internet]. Bethesda (MD): National Institute of Child Health and Human Development; 2006.
2021 May 17.

PMID: 30000954 Bookshelf ID: NBK501894

Free Books & Documents

Excerpt

Evening primrose (*Oenothera biennis*) seed oil contains gamma-linolenic acid (GLA). Oral evening primrose oil (EPO) has been used to treat Raynaud's phenomenon of the nipple in nursing mothers. It is also used for premenstrual syndrome, cyclical mastalgia, and atopic dermatitis. Supplementation of nursing mothers with EPO increases the breastmilk content of linoleic acid and total GLA plus its metabolite, Duomo-gamma-linolenic acid, and caused no adverse reactions in the breastfed infants.[1] Supplementation of mothers with GLA had no effect on the development of atopic dermatitis in their breastfed infants.[2] Evening primrose oil is "generally recognized as safe" (GRAS) as a food by the U.S. Food and Drug Administration.

Heating breastmilk to 63.5 degrees C reduces the concentration of linolenic acid by about 22%. Freezing milk at -20 degrees C and thawing more than once decreases linolenic acid concentration by an average of 63%.[3] Dietary supplements do not require extensive pre-marketing approval from the U.S. Food and Drug Administration. Manufacturers are responsible to ensure the safety, but do not need to *prove* the safety and effectiveness of dietary supplements before they are marketed. Dietary supplements may contain multiple ingredients, and differences are often found between labeled and actual ingredients or their amounts. A manufacturer may contract with an independent organization to verify the quality of a product or its ingredients, but that does *not* certify the safety or effectiveness of a product. Because of the above issues, clinical testing results on one product may not be applicable to other products. More detailed information about dietary supplements is available elsewhere on the LactMed Web site.

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Como usar ?

1. Escolher DOIS suplementos para DERMATITE
2. Pode acrescentar esses dois aos escolhidos de outras imersão
3. Usar na dose indicada do fabricante
4. Utilizar por até 3 meses e rotacionar
5. Pode tomar com chá verde, licorice, boldo, carqueja.

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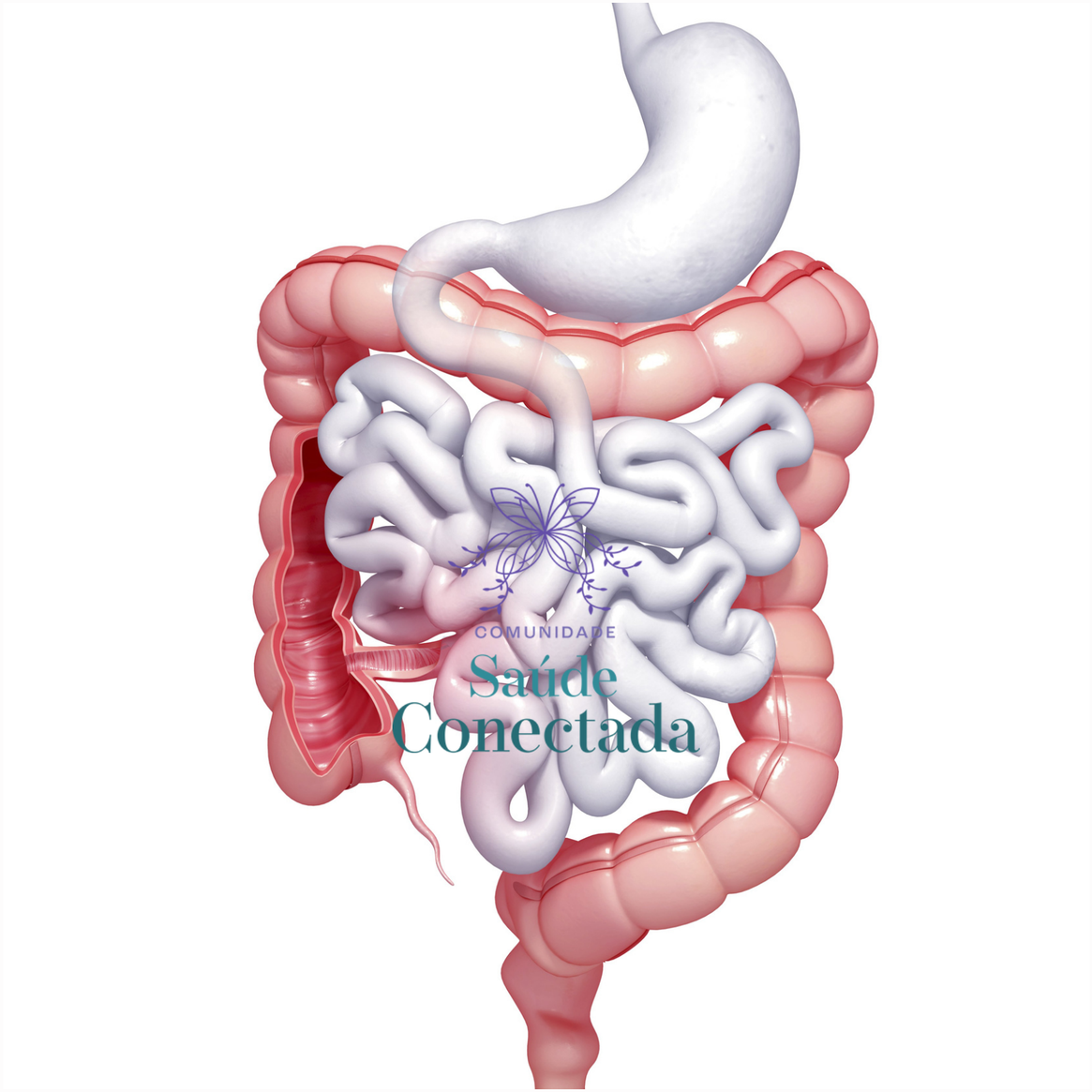

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Review > Front Immunol. 2021 Jul 14;12:720393. doi: 10.3389/fimmu.2021.720393. eCollection 2021.

Gut Microbiota, Probiotics, and Their Interactions in Prevention and Treatment of Atopic Dermatitis: A Review

Zhifeng Fang [1 2], Lingzhi Li [1 2], Hao Zhang [1 2 3 4 5], Jianxin Zhao [1 2 4], Wenwei Lu [1 2 3], Wei Chen [1 2 3]

Affiliations + expand

PMID: 34335634 PMCID: PMC8317022 DOI: 10.3389/fimmu.2021.720393

Free PMC article

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Abstract

Atopic dermatitis (AD) is a public health concern and is increasing in prevalence in urban areas. Recent advances in sequencing technology have demonstrated that the development of AD not only associate with the skin microbiome but gut microbiota. Gut microbiota plays an important role in allergic diseases including AD. The hypothesis of the "gut-skin" axis has been proposed and the cross-talk mechanism between them has been gradually demonstrated in the research. Probiotics contribute to the improvement of the intestinal environment, the balance of immune responses, regulation of metabolic activity. Most studies suggest that probiotic supplements may be an alternative for the prevention and treatment of AD. This study aimed to discuss the effects of probiotics on the clinical manifestation of AD based on gut microbial alterations. Here we reviewed the gut microbial alteration in patients with AD, the association between gut microbiota, epidermal barrier, and toll-like receptors, and the interaction of probiotics and gut microbiota. The potential mechanisms of probiotics on alleviating AD *via* upregulation of epidermal barrier and regulation of immune signaling had been discussed, and their possible effective substances on AD had been explored. This provides the supports for targeting gut microbiota to attenuate AD.

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Microorganisms. 2019 Nov; 7(11): 550.
Published online 2019 Nov 11. doi: 10.3390/microorganisms7110550
PMCID: PMC6920876
PMID: 31717915

The Skin and Gut Microbiome and Its Role in Common Dermatologic Conditions

Samantha R. Ellis,[1,2] Mimi Nguyen,[3] Alexandra R. Vaughn,[2] Manisha Notay,[2] Waqas A. Burney,[2,4] Simran Sandhu,[3] and Raja K. Sivamani[2,4,5,6,7,*]

Author information Article notes Copyright and License information Disclaimer

Abstract

Go to:

Microorganisms inhabit various areas of the body, including the gut and skin, and are important in maintaining homeostasis. Changes to the normal microflora due to genetic or environmental factors can contribute to the development of various disease states. In this review, we will discuss the relationship between the gut and skin microbiome and various dermatological diseases including acne, psoriasis, rosacea, and atopic dermatitis. In addition, we will discuss the impact of treatment on the microbiome and the role of probiotics.

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Research Article

Effects of Oral Administration of *Lactiplantibacillus Plantarum* APsulloc 331261 (GTB1™) Isolated from Green Tea on Atopic Dermatitis (AD)-like Skin Lesion Mouse Models

Su-Young Kim,[1,2] **Jung Ok Lee**,[1] **Yu-Jin Kim**,[1,2] **You Na Jang**,[1,2] **Jung Min Lee**,[1,2] **A. Yeon Park**,[1,2] **Kwang-Ho Yoo**,[1] **and Beom Joon Kim**[1,2]

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Received 18 May 2022; Revised 25 July 2022; Accepted 4 August 2022; Published 6 September 2022

Academic Editor: Jianli Jenny Gao

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Background. Probiotics are known to improve atopic dermatitis (AD) by inhibiting T helper 2 (Th2)-related reactions, restoring the Th2/T helper1 (Th1) cytokine ratio. The most popular probiotic is *Lactiplantibacillus plantarum* (*L. plantarum*), which is widely used in the food and pharmaceutical industries. *L. plantarum* APsulloc 331261 (GTB1) used in this study was isolated from green tea. *Materials and Methods*. The effectiveness of oral GTB1 administration in improving AD was evaluated by visual evaluation, comparison of the lymph node sizes and spleen weights, histological evaluation, RT-qPCR, ELISA, and IHC analysis in the mouse model. *Results*. GTB1 improved AD symptoms, reduced epidermal thickness and mast cell numbers, decreased lymph node size and the spleen weight, increased filaggrin and loricrin protein levels, downregulated Th2 expression, and upregulated Th1 expression in a colony-forming unit-dependent manner. *Conclusion*. Oral administration of GTB1 isolated from green tea (*Camellia sinensis*) improved the AD symptoms, reduced hypersensitivity reaction, and increased the skin barrier function. Finally, it is involved in AD improvement by restoring the Th2/Th1 cytokine balance.

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Review > Exp Ther Med. 2019 Aug;18(2):926-931. doi: 10.3892/etm.2019.7678.
Epub 2019 Jun 14.

Prebiotics and probiotics in atopic dermatitis

Emilia Rusu [1], Georgiana Enache [1 2], Raluca Cursaru [1], Alexandra Alexescu [1], Raluca Radu [1], Oana Onila [1], Teodora Cavallioti [1], Florin Rusu [3], Mihaela Posea [4], Mariana Jinga [5], Gabriela Radulian [1]

Affiliations + expand

PMID: 31384325 PMCID: PMC6639913 DOI: 10.3892/etm.2019.7678

Free PMC article

Abstract

Recent years have brought additional data on the benefits of prebiotics and probiotics treatment in patients with atopic dermatitis. This review includes all the articles published in PubMed, Scopus, Embase and Cochrane databases until 05.06.2018. The terms used for the search were 'prebiotic', 'probiotic', 'atopic dermatitis', 'Severity Scoring of Atopic Dermatitis', and 'SCORAD'. There was an increase of the intestinal permeability reported in patients with atopic dermatitis and a reduction of the cutaneous microbiome diversity. Probiotics modulate the general microbiome and immune status by improving the intestinal barrier; these effects can be responsible for reducing allergic phenomenon and atopic dermatitis severity. We have structured the results by age groups as infants, 1-18 years, adults, and also pregnancy and lactation. Literature does not offer yet answers on the issues such as the optimal dosing, duration needed to see beneficial effects, the optimal time to start the treatment; the personalized use of probiotics according to colonic dysbiosis may be associated with better results. However, most randomized controlled trials and meta-analyzes support the administration of probiotics for at least 8 weeks in order to obtain beneficial effects in improving severity scoring of atopic dermatitis.

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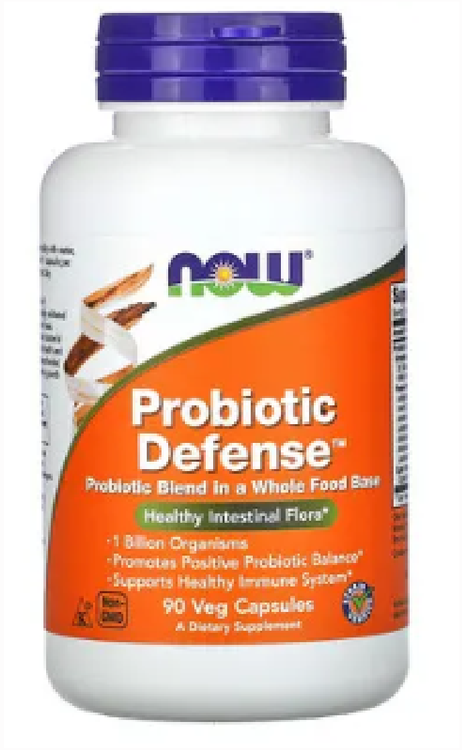

INGREDIENTES

Lactobacillus acidophilus NCFM (ATCC SD5221), Lactobacillus paracasei Lpc-37 (ATCC SD5275), Bifidobacterium animalis subsp lactis BI-04 (ATCC SD5219), Bifidobacterium animalis subsp lactis Bi-07 (ATCC SD5220), agente de massa celulose microcristalina, antiumectantes estearato de magnésio e dióxido de silício, cápsula (espessante hidroxipropilmetilcelulose).

NÃO CONTÉM GLÚTEN.

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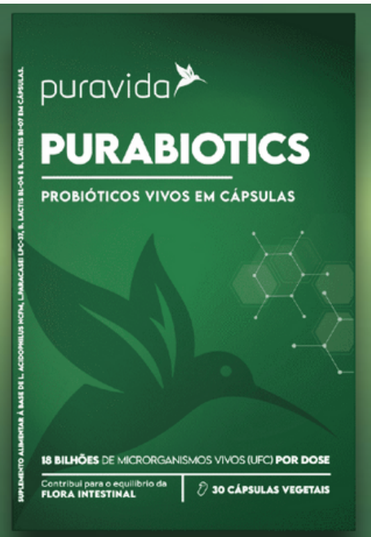
INFORMAÇÃO NUTRICIONAL

Porção de 1,9 g (3 cápsulas)

Quantidade por porção		%VD
Lactobacillus acidophilus NCFM	$4,5x10^9$ UFC	*
Lactobacillus paracasei Lpc-37	$4,5x10^9$ UFC	*
Bifidobacterium animalis subsp lactis BI-04	$4,5x10^9$ UFC	*
Bifidobacterium animalis subsp lactis Bi-07	$4,5x10^9$ UFC	*

Não contém quantidades significativas de valor energético, proteínas, carboidratos, gorduras totais, gorduras saturadas, gorduras trans, fibra alimentar e sódio. *VD não estabelecido.



SUGESTÃO DE USO

ADULTOS (MAIORES DE 19 ANOS), **INGERIR 3 CÁPSULAS AO DIA.**

USO NOTURNO, ANTES DE DORMIR

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CSC

Mascaras naturais

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Mel

Com suas propriedades calmantes, o mel provou ser muito eficaz no tratamento de muitas doenças da pele. Pode ser diluído em água e aplicado na área afetada.
Isso proporciona alívio da coceira, que é muito comum nas bolhas. Isso é especialmente útil na área sob os olhos.

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Este estudo investiga um extrato natural contendo couve crespa, maçã e chá verde rico em antioxidantes por seus efeitos na sinalização de moléculas inflamatórias e no aprimoramento da barreira da pele

O extrato reduz a expressão de citocinas inflamatórias em queratinócitos e aumenta as moléculas relacionadas à barreira. A formulação de verum com altíssima capacidade antioxidante utilizada em pacientes com DA com lesões leves a moderadas reduz o prurido e melhora a função de barreira e a hidratação das lesões cutâneas.

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> Antioxidants (Basel). 2022 May 28;11(6):1071. doi: 10.3390/antiox11061071.

Skin Care Product Rich in Antioxidants and Anti-Inflammatory Natural Compounds Reduces Itching and Inflammation in the Skin of Atopic Dermatitis Patients

Yu Zhang [1], Nina Heinemann [2], Franziska Rademacher [2], Maxim E Darvin [1], Christian Raab [1 3], Cornelia M Keck [3], Henning Vollert [4], Joachim W Fluhr [1 5], Regine Gläser [2], Jürgen Harder [2], Martina C Meinke [1]

Abstract

The atopic dermatitis (AD) complex pathogenesis mechanism reveals marked changes of certain signaling factors as well as some morphological alterations in the epidermis. Reduced resilience against environmental factors and oxidative stress often makes the treatment with corticosteroids or tacrolismus ointments indispensable. In view of the correlation between oxidative stress and AD pathological factors, antioxidants can be incorporated into AD management strategies. This study investigates a curly kale, apple and green tea-containing natural extract rich in antioxidants for its effects on signaling inflammatory molecules and skin barrier enhancement in human epidermal keratinocytes- (NHEKs) based cell assays. Furthermore, the skin penetration on porcine ears was measured ex vivo using Raman micro spectroscopy. Finally, in a double-blind half-side, placebo-controlled clinical study, the effects of a formulation containing this extract were analyzed for the influence of lesion severity, epidermal barrier function, and pruritus in mild to moderately AD patients. Summarizing our results: The extract reduces expression of inflammatory cytokines in keratinocytes and increases barrier-related molecules. The verum formulation with a very high antioxidant capacity used in AD patients with mild to moderate lesions reduces itching, local SCORAD, and improves barrier function and the hydration of skin lesions.

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Aveia

Esmagar uma xícara de aveia e adicioná-la à água do banho ajuda na limpeza da pele e remove a pele morta. Também proporciona alívio contra a irritação, acalma a pele e proporciona alívio da coceira.

Banhos de aveia coloidal são frequentemente recomendados por suas propriedades de cura da pele. A aplicação regular de pasta de aveia nas áreas afetadas ou adicioná-la ao banho pode melhorar a pele afetada pelo eczema. A aveia pode esfoliar suavemente a pele seca e remover as camadas de células mortas, ajudando no processo de renovação da pele.

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Cúrcuma

Uma pasta de óleo de gergelim e açafrão pode ser aplicada na parte afetada.

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Gel de Aloe Vera

Este é um dos tratamentos naturais mais simples e eficazes para o eczema. O gel extraído diretamente da planta de aloe vera pode ser aplicado nas áreas afetadas para alívio imediato. Sua intensa capacidade hidratante hidrata a pele seca e a natureza refrescante acalma a pele afetada por inflamação, eritema e coceira.

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Aloe vera também pode ajudar a reparar a barreira da pele, enquanto suas propriedades antibacterianas ajudam a prevenir as chances de infecção.

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Compressa Fria

Uma compressa fria pode aliviar muitos sintomas de eczema, como inflamação, coceira e vermelhidão. Compressas frias frequentes também podem manter a infecção sob controle.

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Óleo de Coco e Óleo de Semente de Girassol

Tanto o óleo de coco quanto o óleo de semente de girassol são excelentes hidratantes para peles secas e afetadas por eczema. Enquanto o óleo de coco exibe propriedades antibacterianas incríveis, mantendo a infecção afastada, o óleo de girassol ajuda a construir a camada epidérmica danificada. A aplicação regular desses óleos também pode beneficiar os sintomas do eczema, como coceira, inflamação e hiperpigmentação.

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C. Óleo de Jojoba

Jojoba é um excelente óleo pacificador vata dosha. Cura as condições de pele seca resultantes do eczema e ajuda a reconstruir a barreira cutânea danificada. Suas propriedades anti-inflamatórias reduzem a inflamação e acalmam a pele seca e irritada.

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Óleo Vegetal de Jojoba 100ml - Harmonie Aromaterapia

O Óleo Vegetal de Jojoba da Harmonie Aromaterapia é um excelente hidratante de alta absorção, tem ação...

useorganico.com.br /

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1. Massagem com óleo de melaleuca
Um dos óleos mais recomendados para tratar o eczema, o óleo da árvore do chá fornece hidratação para a pele seca. Suas propriedades antibacterianas ajudam a prevenir infecções. Além disso, sua aplicação na pele afetada pelo eczema pode controlar certos gatilhos inflamatórios, devido à presença de flavonoides.

Como usar o óleo da árvore do chá:

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Basta massagear o óleo da árvore do chá nas mãos e nos dedos para curar o eczema das mãos. Também é seguro aplicar para o tratamento de eczema no rosto, ao redor da boca e ao redor dos olhos. Lavar após 20 minutos de aplicação.

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2. Limpeza com chá verde ou preto
Limpar as áreas afetadas com chá é considerado um ótimo remédio caseiro para o eczema. Para o rosto, usar chá verde ou preto como limpador facial pode desobstruir os poros e reduzir as chances de inchaço. Também é um ótimo rejuvenescedor da pele e pode ajudar na prevenção da hiperpigmentação da pele.

Como usar o chá verde ou preto:

Além de tratar o eczema facial, lavar o couro cabeludo com chá pode ajudar a reduzir a descamação, um sintoma comum do eczema do couro cabeludo. Deixe por 20 minutos e lave com água morna.

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4. Manteiga de Karité
Este ingrediente não irritante e amigo da pele pode fornecer hidratação ideal para eczema de pele. Suas propriedades anti-inflamatórias acalmam ainda mais a pele irritada e ajudam a reparar a barreira cutânea.

Como usar manteiga de karité:

A manteiga de karité pode ser usada diariamente como um hidratante para o eczema da pele.

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Calêndula e mel

Flores e folhas de calêndula são medicamentos antigos usados por suas propriedades cicatrizantes, anti-inflamatórias e antimicrobianas. Usá-lo em combinação com mel melhora ainda mais o tratamento do eczema.

Como usar calêndula e mel:

Faça um emplastro de rosto moendo 4 a 5 flores e folhas de calêndula. Adicione o mel à mistura e aplique nas áreas afetadas. Lave após 15 a 20 minutos para curar o eczema leve.

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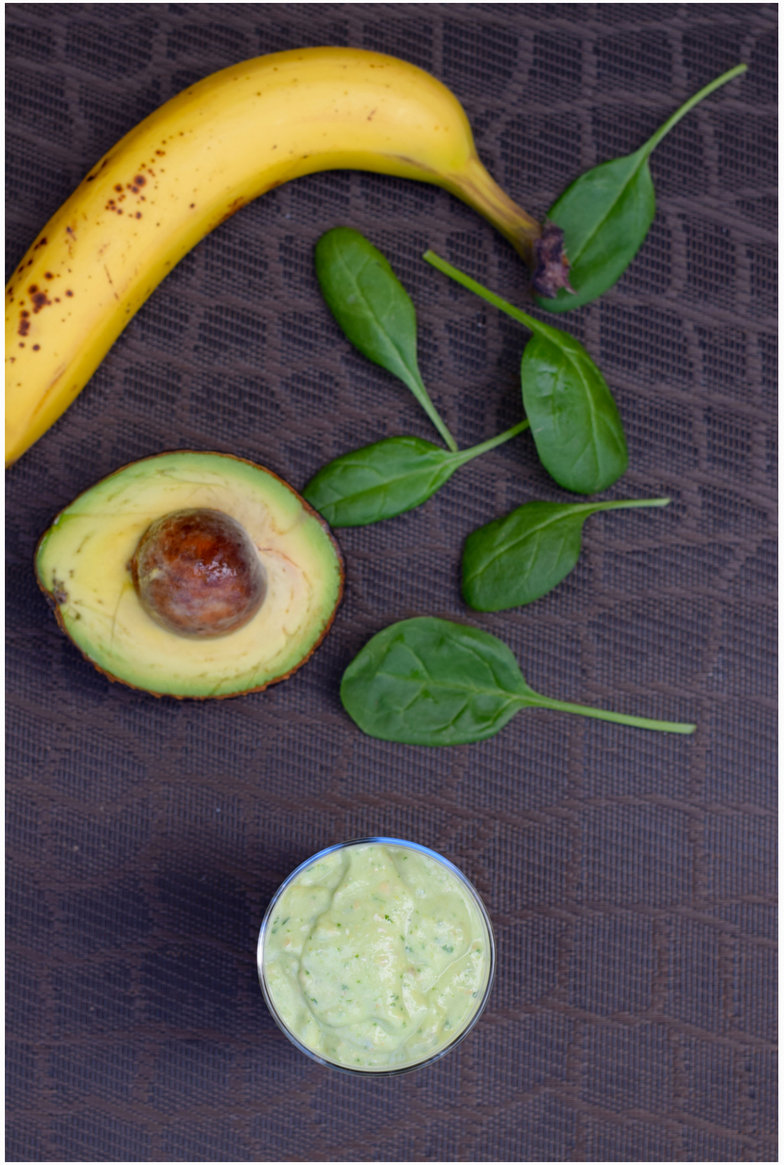
Mix facial de abacate, banana e iogurte

Este é um remédio caseiro incrível para eczema. As habilidades super hidratantes do abacate e da banana acalmam a pele afetada pelo eczema e evitam a perda da umidade essencial. O abacate também é conhecido por suas habilidades anti-inflamatórias e pode suprimir os mediadores inflamatórios desencadeados pelo eczema. A adição de iogurte a este pacote reduz a descamação da pele e evita a caspa no eczema do couro cabeludo.



Como usar:
Misture um abacate totalmente maduro e uma banana e adicione o iogurte. Bata bem e aplique para tratar o eczema da pele. Lavar após 10 a 15 minutos.

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Feno-grego, Curry e Hibiscos

Essas ervas poderosas têm a capacidade de tratar o eczema do couro cabeludo ou a dermatite seborreica. As saponinas suaves do feno-grego e do hibisco ajudam a remover a caspa do couro cabeludo. Tanto o curry quanto o hibisco promovem o crescimento do cabelo e previnem a queda de cabelo devido ao eczema do couro cabeludo. Todos os três nutrem o couro cabeludo e previnem sintomas como ressecamento, coceira e infecção.

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Como usar:
Mergulhe as sementes de feno-grego durante a noite e faça uma pasta adicionando 7 a 8 flores de hibisco, 4-5 folhas de hibisco e um punhado de curry. Aplique isso no couro cabeludo e lave após 20 minutos.

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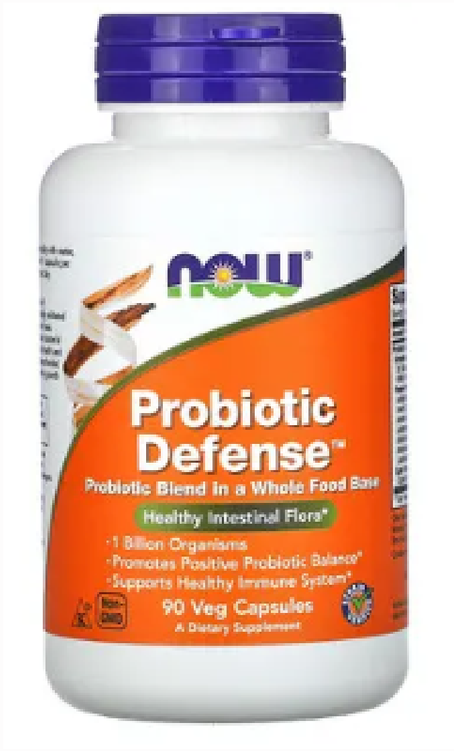
Exemplo de suplementação

Onde comprar?
Site das marcas
Mercado Livre (ver pontuação do vendedor)
Amazon
Iherb



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